

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient has a medium risk for preeclampsia, primarily driven by elevated systolic blood pressure (142 mmHg) and positive proteinuria (1), warranting further medical evaluation. She also presents with a medium risk for macrosomia, influenced by factors such as high pre-pregnancy BMI and maternal age, despite current glucose levels within general normal ranges and some positive lifestyle choices.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: Patient TEST_MED_MED, 32 weeks gestational age, presents with a medium preeclampsia risk tier. Current vitals include systolic BP 142 mmHg, diastolic BP 79 mmHg, and proteinuria dipstick 1. Patient reports no severe headache, vision changes, upper abdominal pain, or swelling of face/hands. Also noted is a medium macrosomia risk tier, influenced by pre-pregnancy BMI 42.4 and maternal age 38, with fasting glucose 99 mg/dL and 1hr postprandial glucose 126 mg/dL.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

